

Patient ID: JonesPeriod: April 1 to April 30, 2026Report Date: May 1, 2026Coverage: 510/720h (71%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Last 24 Hours

Last 24 Hours: within normal range			
Vital Stat	Avg	Min	Max
Heart Rate	55 bpm	44 bpm	73 bpm
Breathing	17 brpm	11 brpm	29 brpm

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

Patient clinical status over 30 days from Apr 01 to Apr 30

HRBreathing

Elevated Breathing Apr 09 to Apr 09 (1d)	High Breathing Apr 13 to Apr 13 (1d)	Low HR Apr 22 to Apr 22 (1d)	Low HR Apr 27 to Apr 27 (1d)
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5 episodic events Detected, spanning 5h total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
5	5h	Elevated Breathing	<1	71%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 13	11 PM to 12 AM	1h	1	High Breathing	31	8	48	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Apr 22	4 AM to 5 AM	1h	1	Low Heart Rate	44	42	47	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 27	11 PM to 12 AM	1h	1	Low Heart Rate	44	42	50	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Apr 09	2 PM to 3 PM	2h	2	Elevated Breathing	26	13	35	Check SpO2 and lung sounds; assess for fluid overload or early infection

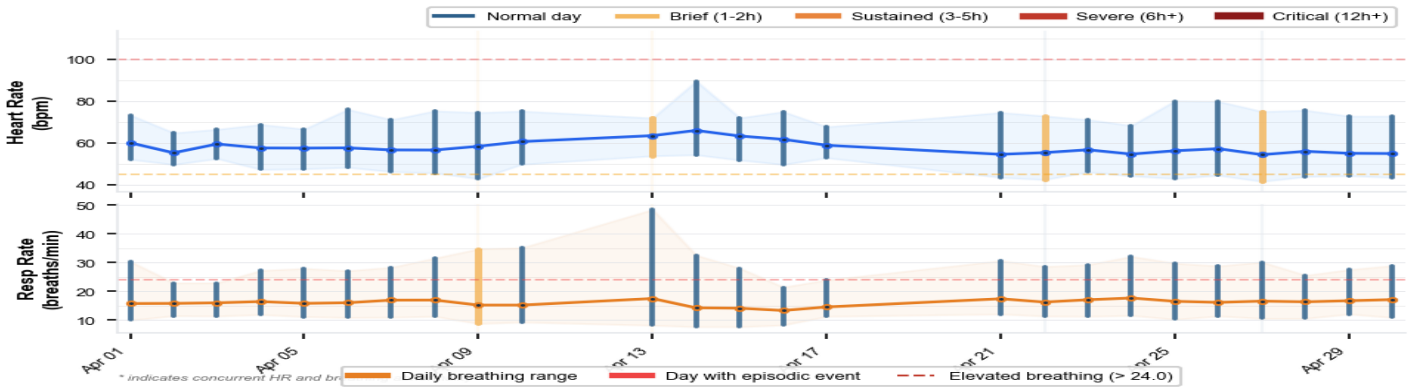
30% of readings are low confidence.

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 35 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 31 brpm, peak 48 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Low Heart Rate (avg 44 bpm, min 42 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 4 unstable phases with 14 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

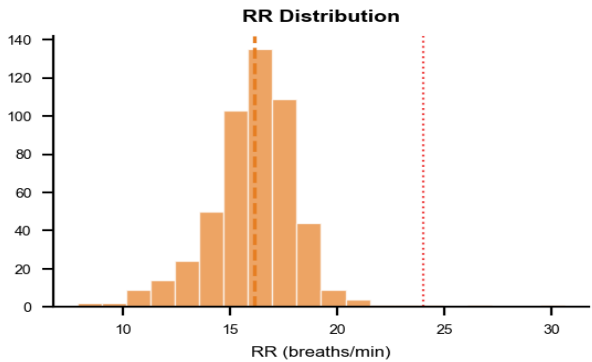
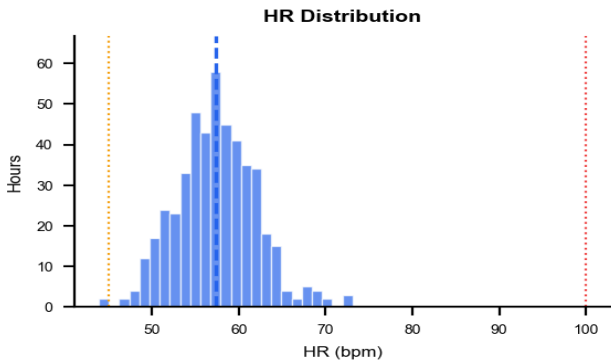
Clinical Pattern Observations:

- 1. **Clustered pattern:** Episodic events on 4 of 30 days (13%).
- 2. **Nocturnal pattern:** 3 of 5 eligible episodes during evening or night hours.

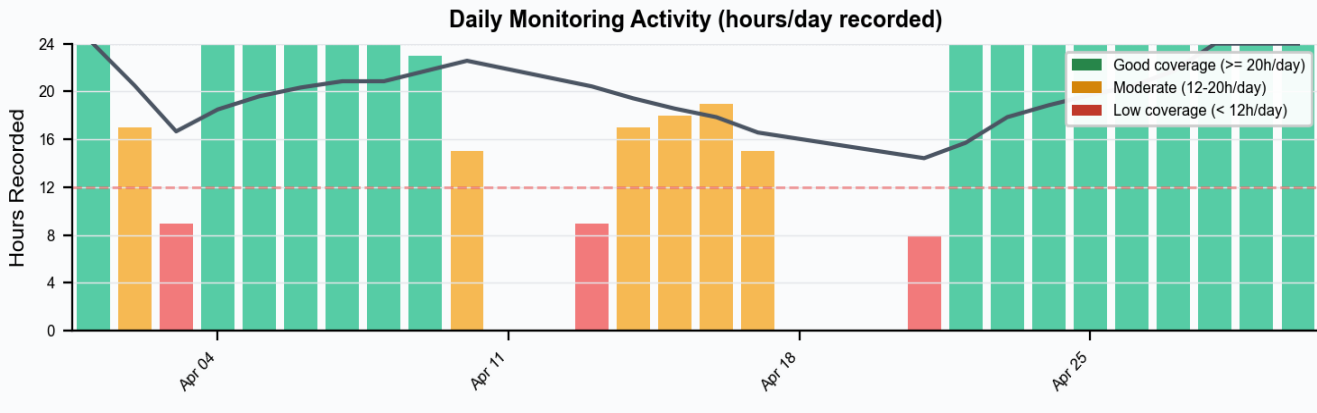


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.